

CHAPTER 21

Appropriate oral drugs

In **Part IV** you learned how to identify the treatment needed for sick young infants and children. Sick children often begin treatment at a clinic and need to continue treatment at home. The *TREAT THE CHILD* chart and the TREAT THE YOUNG INFANT AND COUNSEL THE MOTHER section of the *YOUNG INFANT* chart describe how to give the treatments needed. Use these charts to select the appropriate drug, and to determine the dose and schedule.

The success of home treatment depends on how well you communicate with the child's mother or caretaker. She needs to know how to give the treatment. She also needs to understand the importance of the treatment. Part VI describes good communication skills and gives information needed to teach the mother or caretaker to care for the young infant or child at home.

There are some important points to remember about each oral drug.

21.1 Oral antibiotics

The following classifications need an oral antibiotic.

For **sick young infants** age 1 week up to 2 months:

- LOCAL BACTERIAL INFECTION
- DYSENTERY

For **sick children** age 2 months up to 5 years:

- SEVERE PNEUMONIA OR VERY SEVERE DISEASE
- PNEUMONIA
- SEVERE DEHYDRATION with cholera in the area
- DYSENTERY
- VERY SEVERE FEBRILE DISEASE
- SEVERE COMPLICATED MEASLES
- MASTOIDITIS
- ACUTE EAR INFECTION

In many health facilities more than one type of antibiotic will be available. You must learn to **select the most appropriate antibiotic** for the child's illness. If the child is able to drink, give an oral antibiotic. Give the "first-line" oral antibiotic if it is available. It has been chosen because it is effective, easy to give and inexpensive. Recommended first-line and second-line antibiotics may need to be changed based on resistance data. You should give the "second-line" antibiotic only if the first-line antibiotic is not available, or if the child's illness does not respond to the first-line antibiotic.

Some children have more than one illness that requires antibiotic treatment. Whenever possible, select one antibiotic that can treat all of the child's illnesses.

- **Sometimes one antibiotic can be given to treat the illnesses.**

For example, a child with PNEUMONIA and ACUTE EAR INFECTION can be treated with a single antibiotic. A child with DYSENTERY and ACUTE EAR INFECTION can be treated with cotrimoxazole if the first-line antibiotic for an ACUTE EAR INFECTION (cotrimoxazole) is also a first- or second-line antibiotic for DYSENTERY.

When treating a child with more than one illness requiring the same antibiotic, do not double the size of each dose or give the antibiotic for a longer period of time.

■ **Sometimes more than one antibiotic must be given to treat the illnesses.**

For example, the antibiotics used to treat PNEUMONIA may not be effective against DYSENTERY in your country. In this situation, a child who needs treatment for DYSENTERY *and* PNEUMONIA must be treated with two antibiotics.

The TREAT charts show the schedule for giving the antibiotic and the correct dose of the antibiotic to give to the young infant or child (see Example 25). The **schedule** tells *how many days* and *how many times each day* to give the antibiotic. Most antibiotics should be given for 5 days. Only cholera cases receive antibiotics for 3 days. The number of times to give the antibiotic each day varies (2, 3 or 4 times per day).

To determine the correct **dose** of the antibiotic:

- Refer to the column that lists the concentration of tablets or syrup available in your clinic.
- Choose the row for the child's weight or age. The weight is better than the age when choosing the correct dose. The correct dose is listed at the intersection of the column and row.

EXAMPLE 25: TOP PART OF A SCHEDULE AND DOSE TABLE FOR ORAL ANTIBIOTICS

► Give an Appropriate Oral Antibiotic ► FOR PNEUMONIA, ACUTE EAR INFECTION OR VERY SEVERE DISEASE: FIRST-LINE ANTIBIOTIC: COTRIMOXAZOLE SECOND-LINE ANTIBIOTIC: AMOXYCILLIN				
Schedule for Cotrimoxazole	COTRIMOXAZOLE ► Give two times daily for 5 days			AMOXYCILLIN ► Give three times daily for 5 days
	AGE OR WEIGHT	ADULT TABLET 80 mg trimethoprim + 400 mg sulphamethoxazole	PEDIATRIC TABLET 20 mg trimethoprim + 100 mg sulphamethoxazole	SYRUP/per 5 ml 40 mg trimethoprim + 200 mg sulphamethoxazole
	2 months up to 12 months (4–<10 kg)	1/2	2	5 ml
	12 months up to 5 years (10–<19 kg)	1	3	7,5 ml

Dose of syrup for a child age 2 months up to 12 months (4–10 kg)

Note: Avoid giving cotrimoxazole to a young infant less than 1 month of age who is premature or jaundiced. Give this infant amoxycillin or benzylpenicillin instead.

21.2 Oral antimalarials

Oral antimalarials vary by country. Chloroquine and sulfadoxine-pyrimethamine are the first-line and second-line drugs used in many countries. The first- and second-line oral antimalarials recommended in your country are on the TREAT chart. It may be that only the first-line antimalarial is available at your clinic.

Refer to the *TREAT THE CHILD* chart to determine the dose and schedule for an oral antimalarial. There are a few important points to remember about giving oral antimalarials:

- Treatment with chloroquine assumes that the child has not already been treated with chloroquine. Confirm this with the mother. Ask her if her child has already been given a full course of chloroquine for this fever. If so, and the child still has fever, consider this a follow-up visit. Use the instructions in the box “GIVE FOLLOW-UP CARE—MALARIA” on the *TREAT THE CHILD* chart.
- Chloroquine is given for 3 days. The dose is reduced on the third day **unless** the child weighs less than 10 kg and you are giving 150 mg base chloroquine tablets. In this case, the child is given the same dose (that is, 1/2 tablet) on all 3 days.
- Cotrimoxazole may be used both as antibiotic and as antimalarial. It is effective against *P. falciparum* malaria in children under 5 years of age if given for 5 days.
Note: Amoxycillin and chloramphenicol do not work against malaria. In many countries, cotrimoxazole works because, like sulfadoxine-pyrimethamine, it is a combination of two antifolate drugs that are effective against the falciparum malaria parasite.
- Explain to the mother that itching is a possible side effect of chloroquine. It is not dangerous. The mother should continue giving the drug. The child does not need to return to the clinic because he is itching.

21.3 Paracetamol for high fever (>38.5 °C) or ear pain

Paracetamol lowers a fever and reduces pain. If a child has high fever, give one dose of paracetamol in clinic.

If the child has ear pain, give the mother enough paracetamol for 1 day, that is, 4 doses. Tell her to give one dose every 6 hours or until the ear pain is gone.

21.4 Vitamin A

Vitamin A is given to a child with MEASLES or SEVERE MALNUTRITION. Vitamin A helps resist the measles virus infection in the eye as well as in the layer of cells that line the lung, gut, mouth and throat. It may also help the immune system to prevent other infections. Corneal clouding, a sign of vitamin A deficiency can progress to blindness if vitamin A is not given.

Vitamin A is available in capsule and syrup. Use the child's age to determine the dose. Give 2 doses. Give the first dose to the child in the clinic. Give the second dose to the mother to give her child the next day at home. If the vitamin A in your clinic is in capsule form, make sure the child swallows it whole. If the child is not able to swallow a whole capsule or needs only part of the capsule, open the capsule. Tear off or cut across the nipple with a clean tool. If the vitamin A capsule does not have a nipple, pierce the capsule with a needle.

Record the date each time you give vitamin A to a child. This is important. If you give repeated doses of vitamin A in a short period of time, there is danger of an overdose.

21.5 Iron

A child with some palmar pallor may have anaemia. A child with anaemia needs iron.

Give syrup to the child under 12 months of age. If the child is 12 months or older, give iron tablets. Give the mother enough iron for 14 days. Tell her to give her child one dose daily for the next 14 days. Ask her to return for more iron in 14 days. Also tell her that the iron may make the child's stools black.

Tell the mother to keep the iron out of reach of the child. An overdose of iron can be fatal or make the child very ill.

If a child with some pallor is receiving the antimalarial sulfadoxine-pyrimethamine (Fansidar), do **not** give iron/folate tablets until a follow-up visit in 2 weeks. The iron/folate may interfere with the action of the sulfadoxine-pyrimethamine that contains antifolate drugs. If the iron syrup at your clinic does not contain folate, you can give the child iron syrup with sulfadoxine-pyrimethamine.

21.6 Mebendazole

If hookworm or whipworm is a problem in your area, an anaemic child who is 2 years of age or older, needs mebendazole. Mebendazole treats hookworm and whipworm infections. These infections contribute to anaemia because of iron loss through intestinal bleeding.

Give 500 mg mebendazole as a single dose in the clinic. Give either one 500 mg tablet or five 100 mg tablets.